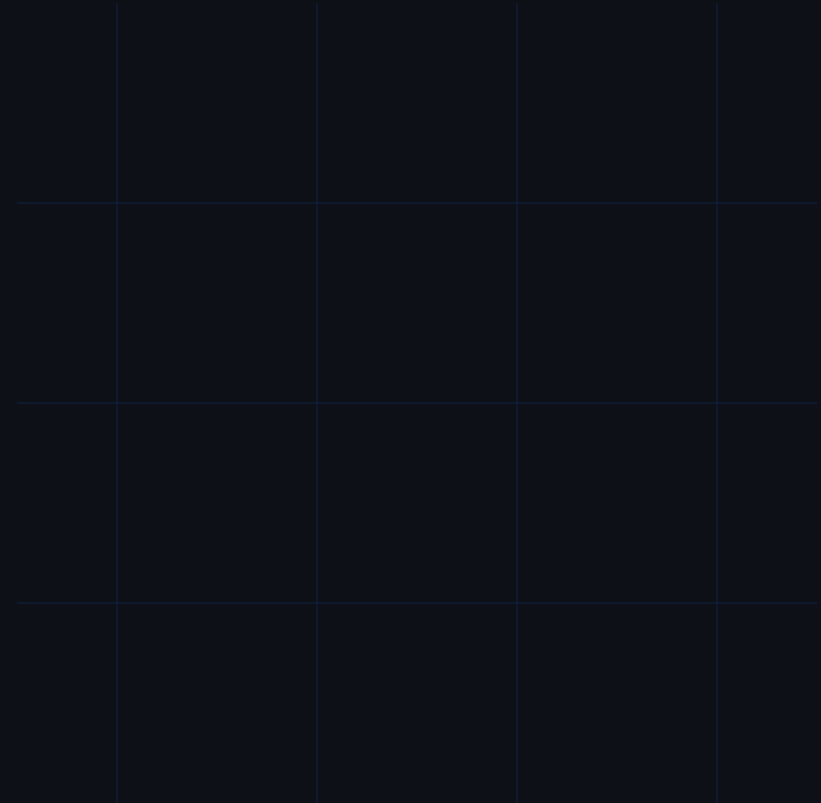




The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

A DATA-DRIVEN POLICY BRIEFING



Hundreds of Millions Affected



1 in 3

Women

will experience major depression
in their lifetime



1 in 5

Men

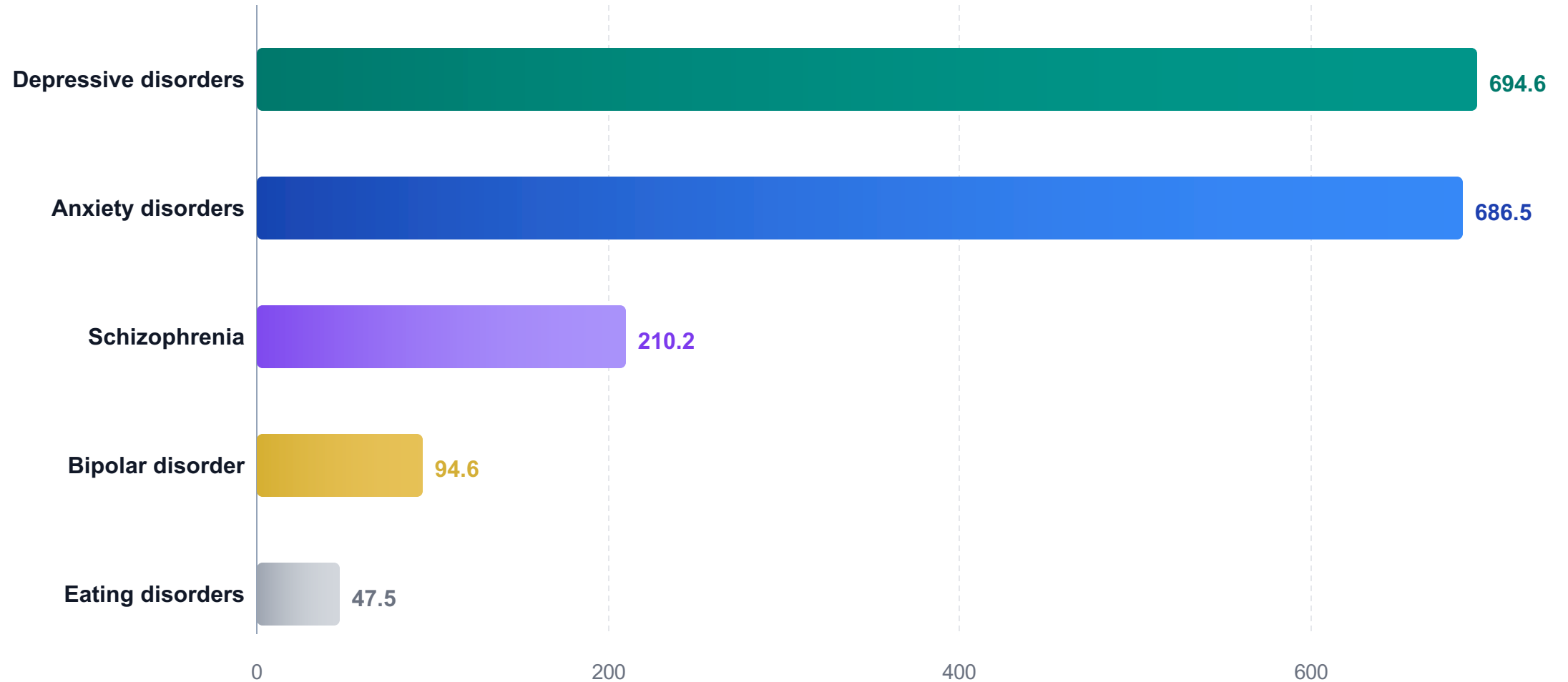
will experience major depression
in their lifetime

Mental health conditions span from common disorders — depression and anxiety affect hundreds of millions each year — to less common but severe conditions like schizophrenia and bipolar disorder that carry a large impact on individuals' lives.

Poor mental health affects well-being, the ability to work, and relationships with family and community.

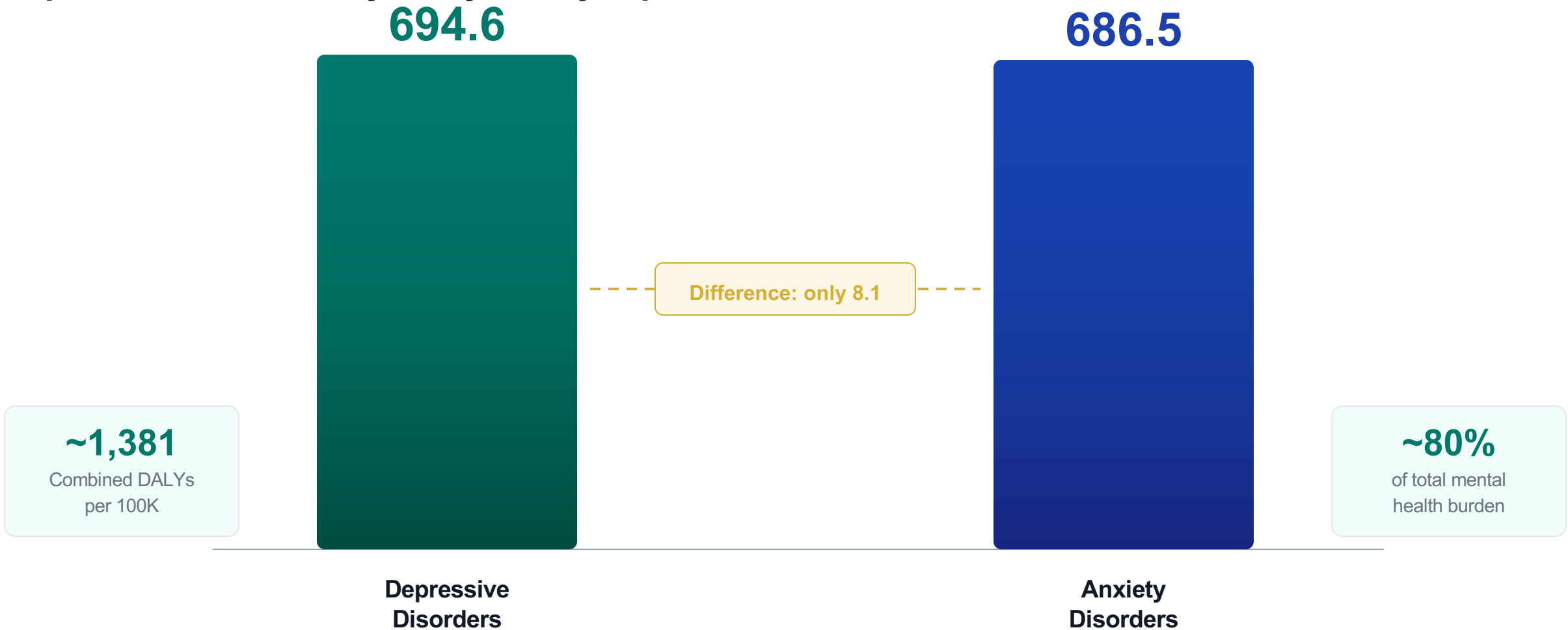
Together, depression and anxiety account for ~80% of the total mental health disease burden

Disease Burden by Category — DALYs per 100,000 People (World, 2023)



Near-parity: ~695 vs ~687 DALYs per 100K — combined, they account for ~80% of total burden

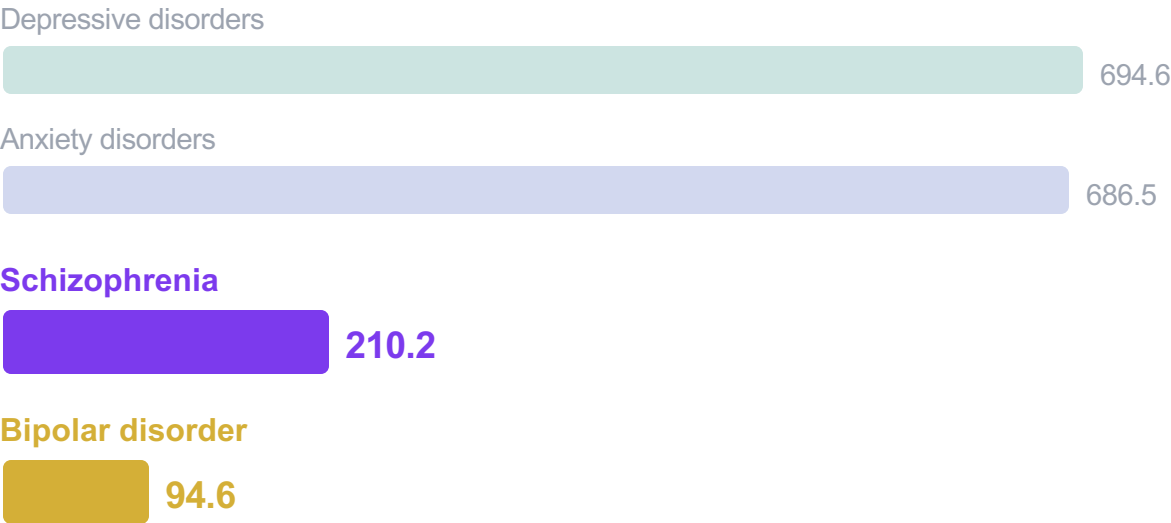
Depression and Anxiety Carry Nearly Equal Disease Burden Worldwide



Schizophrenia at 210.2 DALYs/100K reflects disproportionately high disability per case

Schizophrenia and Bipolar: Lower Prevalence but Severe Individual Impact

RELATIVE SCALE (DALYs per 100K)



Schizophrenia: 210.2 DALYs

While affecting fewer people, this condition reflects severe disability per individual case. Requires sustained, specialized clinical care.



Bipolar disorder: 94.6 DALYs

A meaningful burden — individuals face episodic disability requiring ongoing management.

Policy must not neglect severe conditions while scaling interventions for common disorders.

Treatment Is Available but Often Lacking or Poor Quality



Stigma

Many people are uncomfortable disclosing symptoms to healthcare professionals or even to people they know.



Access

Treatment is often lacking, especially in low- and middle-income countries where resources are scarce.



Quality

Even where treatment exists, it may be of poor quality, failing to meet the needs of patients effectively.



Underreporting

Discomfort with disclosure makes it difficult to estimate true prevalence — the actual burden may be higher than reported.

Good data is essential to understand these conditions and treat them effectively and safely.

How People Cope: From Medication to Faith to Family Conversations



Prescribed Medication

Common in higher-income settings where healthcare access is more established.

Higher-income countries



Religious / Spiritual Activities

Prevalent in many regions worldwide as a primary coping mechanism.

Globally prevalent



Talking to Friends or Family

A widely reported coping mechanism across diverse cultural contexts.

Cross-cultural



Policy interventions must account for cultural diversity — no one-size-fits-all approach will work.

Key Takeaways: Prioritizing Depression, Anxiety, and Closing the Treatment Gap

1

Depression + anxiety together represent the overwhelming majority of mental health DALYs

~694.6 and ~686.5 DALYs per 100K respectively — combined ~80% of total burden

2

Schizophrenia + bipolar carry disproportionate individual burden

Schizophrenia: 210.2 DALYs/100K — severe disability despite lower population prevalence

3

Treatment exists but access and quality remain inadequate

Especially in low- and middle-income countries — effective treatments are available but not reaching those in need

4

Stigma suppresses both help-seeking and accurate data

Underreporting means the true burden is likely higher than current estimates suggest

5

**Policy should prioritize scaling evidence-based interventions for common disorders
while maintaining specialized care for severe conditions like schizophrenia and bipolar disorder**